

STEADY — AUTONOMOUS SAFETY SYSTEM**Bilateral Stimulation (BLS) Protocol — Clinician Sign-Off**

Protocol version: bls-protocol-v1-DRAFT Date: 07/22/2026 Ref: STEADY-BLS-CLINREV-2026-07-22

Why this sheet exists. The clinical review of config beta-clinrev-2026-07 explicitly did NOT ratify autonomous bilateral stimulation (ledger A7: “not ratified — nothing ships without this row”). Autonomous BLS / trauma-memory reprocessing is DISABLED in beta and stays disabled until this protocol is defined and signed. This sheet is that gate.

Scope. Signing authorizes ONLY the specific, self-administered BLS protocol defined here — no live clinician is present during the session. It does not authorize general reprocessing, and it does not take effect until the Part 6 validation gates are complete. The deterministic session engine still owns every clinical stop/closure decision; this defines the parameters and the words within those rails.

**Part 1 — Reviewer identity & credentials**

At least two independent licensed trauma clinicians must sign (Vol I App. C). Signatures applied personally in Part 7.

Reviewer 1	
Full name	Rebecca Altschuler
Professional title	Licensed Psychologist
License type & number	Psychologist — PSY-005804 (Active, exp. 02/29/2028; no board actions)
Jurisdiction / state	Arizona (Phoenix)
EMDR training / certification	EMDRIA-trained; EMDRIA certification/training confirmed by reviewer
Trauma-clinical experience	Trauma-focused clinical experience confirmed by reviewer
Independence attestation	I attest that I am acting as an independent licensed clinician and have no financial or operational conflict that would impair my clinical judgment regarding this protocol.

Reviewer 2	
Full name	John Allen
Professional title	Licensed Psychologist
License type & number	Psychologist — PSY-002055 (Active, exp. 08/31/2027; no board actions)
Jurisdiction / state	Arizona (Tucson)
EMDR training / certification	EMDRIA-trained; EMDRIA certification/training confirmed by reviewer
Trauma-clinical experience	Trauma-focused clinical experience confirmed by reviewer
Independence attestation	I attest that I am acting as an independent licensed clinician and have no financial or operational conflict that would impair my clinical judgment regarding this protocol.

**Part 2 — BLS parameters (define the authoritative value for each)**

For each parameter: the current code placeholder and a common EMDR reference are shown. Enter the authoritative value you approve for a self-administered protocol (or Approve the placeholder). Reference values are for context only and require your confirmation — they are not clinical direction.

Parameter	Current placeholder (in code)	Common EMDR reference (confirm)	Authoritative value you approve
Modality permitted (beta)	Auditory + self-tapping only; visual BLS OFF (BLS_NO_VISUAL_BETA)	Auditory tones / tactile taps are standard alternatives to eye movement; visual requires a11y validation	APPROVED AS WRITTEN
Visual flash ceiling (if ever visual)	≤ 3 flashes/sec (WCAG 2.3.2) — hard ceiling	Photosensitivity safety limit; not a clinical dosing value	APPROVED AS WRITTEN
Speed (Hz)	default 1.25 Hz; range 1.0–1.5; cautious 1.0	~1 alternation/sec is common; faster used by some clinicians	APPROVED AS WRITTEN
Desensitization set length	20–30 s per set (module content)	~24–30 passes standard; clinicians LENGTHEN while actively processing	APPROVED AS WRITTEN
Resourcing / Calm-Place set length	not yet distinct from above	SHORT slow sets (~4–8 passes) so it does NOT open processing	APPROVED AS

			WRITTEN
Max sets per session (beta)	2 (SESSION.maxSets / BETA_CONFIG)	Beta-conservative; standard sessions run many more with a clinician	APPROVED AS WRITTEN
Extend-vs-stop policy per set	fixed length; no adaptive extension (no clinician reading client)	Clinicians extend on active processing, stop at a natural pause — needs an unsupervised-safe rule	APPROVED AS WRITTEN
Starting-SUDS gate to permit a set	> 5 denies stimulation (SESSION.startingSudsCeiling)	Judgment-based clinically; a fixed ceiling is the unsupervised substitute	APPROVED AS WRITTEN
Between-set procedure	collect SUDS + dissociation + orientation; then reassess	"What do you get now?" + brief check; the responsive window	APPROVED AS WRITTEN
Containment triggers (auto-stop)	SUDS $\Delta \geq 2$, or ≥ 8 absolute, or rise ≥ 3 over start, or 2 consecutive +1s, or dissociation ≥ 4 , or not oriented	Conservative union — auto fail-safes, not a substitute for clinical read	APPROVED AS WRITTEN
"Stuck" rule	no SUDS change across 2 sets → close ("stuck is a stop signal")	Confirm the count and the interpretation	APPROVED AS WRITTEN
Session time limits	wind-down (no new sets) 30 min; hard-stop 40 min	Confirm for an unsupervised processing session	APPROVED AS WRITTEN
Closure minimum + required content	≥ 120 s; PLUS orientation confirmation + member-reported stability + escalation path on failure	Closure is mandatory regardless of SUDS; define the exact required content	APPROVED AS WRITTEN
Incomplete-session / container	containment → cooldown; container exercise not yet specified	Define the incomplete-target procedure + container the system uses	APPROVED AS WRITTEN
Cooldown after containment-ending	48 h (COOLDOWN_HOURS.containmentEnding)	Minimum rest + fresh check-in; interval pending evidence (ledger A4)	APPROVED AS WRITTEN



Part 3 — Verbal facilitation model

Proposed design: light + directive DURING a set (short pre-scripted cues on a fixed cadence — e.g. “go with that,” “just notice,” “breathe” — deterministic, not listening/reacting mid-set), and responsive check-ins only BETWEEN sets and at open/close. No conversational AI runs while stimulation is active.

Directive during-set cue bank	Approve a fixed, clinician-authored list of short cues the system may speak during a set. It must be deterministic and output-guarded (no reprocessing instructions).	☒ Approve ☐ Needs-change _____
Between-set check-in wording	Approve the between-set prompt(s) (e.g. “What do you notice now?”) and how a numeric SUDS is collected.	☒ Approve ☐ Needs-change _____
Cognitive interweaves	Approve whether/what interweaves the system may offer if processing loops or stalls — or require it to close/ground instead (no interweave without a clinician).	☒ Approve ☐ Needs-change _____
“Never say” list	Confirm the banned outputs: worst-memory recall, sustained imagery, “stay with it until it drops,” simulated feelings, monitoring claims.	☒ Approve ☐ Needs-change _____

Required changes / approved cue text: None. Proposed deterministic cue bank and between-set wording approved as written; no autonomous cognitive interweaves.



Part 4 — Contraindications & consent

Confirm who may NEVER do autonomous BLS, and the consent required. Mark Approve or write the correct rule.

☒ Exclusions: high dissociation (e.g. DES-II threshold — DES-II currently omitted in beta), acute trauma window, standing restrictions (psychotic/dissociative dx, recent hospitalization, substance dependence pending human review), and anyone flagged not-safe today.

☒ A distinct, versioned PROCESSING-SESSION consent is required before any BLS set (separate from care-program + voice consents), disclosing that no live clinician is present and no real-time monitoring occurs.

☒ The member may stop at any time; stopping early is never penalized and is stated up front.

Corrections: None. Approved as written.

Part 5 — Self-administered safety constraints (confirm each is adequate)

☒ No live clinician is reading the client — the system must NOT extend a set based on inferred processing; set length is fixed/conservative.

☒ Ground-Me: a one-tap immediate halt is reachable at all times, locks stimulation for the session, no return.

☒ Loss of present orientation or inability to follow a stop instruction is an absolute hard stop (overrides SUDS).

☒ Every spoken line is deterministic or output-guarded: never instructs worst-memory recall / sustained imagery /

- "stay with it until it drops"; never interprets a falling SUDS as improvement when dissociation is possible.
- ☒ Crisis / high-activation input is scripted to present-safety + jurisdiction-aware resources with truthful notification status — never AI-generated, never implies live monitoring.
 - ☒ Mandatory closure runs even on an incomplete/aborted session; the session cannot silently end mid-processing.
 - ☒ A network/timing failure stops the set (never catches up or resumes) and routes to grounding + closure.
- ☐

Part 6 — Validation gates before any real-member BLS session

None of these is waived by this sign-off; the protocol may not run for a real member until all are complete + documented.

Gate	Status + evidence reference
Independent clinical review of this protocol by ≥ 2 licensed trauma clinicians (this sheet).	☒ Complete ☐ Pending Ref: STEADY-BLS-CLINREV-2026-07-22
Human-factors testing specifically on the BLS session flow (stop-control salience, interruption recovery, distress de-escalation, comprehension).	☐ Complete ☒ Pending Ref: To be completed before any real-member BLS session
Red-team closure on BLS paths: abreaction, dissociation mid-set, network drop mid-set, stop reachability, output-guard on cues.	☐ Complete ☒ Pending Ref: To be completed before any real-member BLS session
Distinct, versioned PROCESSING-SESSION consent (separate from the care-program and voice consents) + counsel review.	☐ Complete ☒ Pending Ref: To be completed before any real-member BLS session
Staged Phase-4 rollout with predefined progression + stopping criteria and a kill switch.	☐ Complete ☒ Pending Ref: To be completed before any real-member BLS session

☐

Part 7 — Attestation & signatures

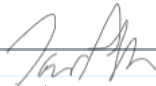
By signing, each reviewer attests that: they are an independent, licensed trauma clinician; they have defined/confirmed the BLS parameters, verbal model, contraindications, and safety constraints above for a SELF-ADMINISTERED protocol with no live clinician present; and that no autonomous BLS may run for a real member until the Part 6 gates are complete. This authorization applies only to protocol version **bls-protocol-v1-DRAFT**; any change to an approved parameter voids it and requires renewed review. Signatures must be applied personally by the named reviewers.

Overall determination:

- ☒ Protocol APPROVED for validation (Part 6) — all parameters set, no blocking Needs-change.
- ☐ Approved WITH CONDITIONS (list below).
- ☐ NOT approved — autonomous BLS remains disabled.

Conditions / notes: Protocol approved as written for validation only. No real-member BLS session may occur until every Part 6 validation gate is complete and documented.

 Reviewer 1 — signature	Rebecca Altschuler / Psychologist — PSY-005804 Printed name / license #
7/22/26 Date	

 Reviewer 2 — signature	John Allen / Psychologist — PSY-002055 Printed name / license #
7/22/26 Date	

